

Seizure

History

- How many seizures? How long did they last?
- What was patient doing when seizure started?
- Did the patient fall? Any injuries?
- Does patient have a history of seizures? What is the cause? How long has patient had seizures?
- Seizure medications? Missed dose(s)?
- Alcohol or drug ingestion?
- Fever? (Especially in children)
- Recent head injury? Recent headache?
- Sleep deprivation? Emotional upset?

Physical

- Is patient post-ictal? Actively seizing?
- Is patient maintaining airway?
- Vital signs – Hypertension? Hypotension? Tachycardia? Hypoxia?
- Blood glucose
- Signs of trauma?
- Signs of stroke?
- Urinary / fecal incontinence?
- Signs of drug or alcohol intoxication?
- Eye Exam: Nystagmus or Disconjugate gaze?

Note:

Status Epilepticus or Status seizure is defined as:

- Continuous seizure lasting longer than 5 minutes OR
- Two consecutive seizures without resolution of altered mental status

EMT/AEMT

- Follow General Medical Care Guideline
- Follow Adult Resuscitation Guideline, as indicated

Paramedic

Actively seizing

- **Levetiracetam** Loading dose of 40-60mg/kg IV over 5-15 minutes
 - Max dose: 4.5g
 - Maintenance dose: 1-1.5g over 15 minutes every 12 hours

Post-Ictal

- Airway support as indicated with capnography
- **NS/LR** 500 – 1000 mL IV/IO, per Medical Shock Care Guideline
- Recheck Blood Glucose, if indicated
- 12 – Lead EKG
- Consider giving loading dose of Levetiracetam while arranging evacuation

When Awake and oriented

- If missed dose of home anti-epileptic - may assist patient in taking this dose
- If no prior seizures - attempt to determine possible cause and evacuate